PRESENTATION SCRIPT | FIVE-MINUTE PROPOSAL DEFENSE

MAP - A

A Landmark-Anchored Timetable-Graph Model of Public Transport: Verified Computation of Accessibility, Its Inequality, and the Fare Matrix from Open Data, with Agent-Reported Hospital Bed Capacity for Ambulance and Patient Routing

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Delivery notes

- Pace: about 135 words a minute, slower than conversation. With the pauses, the full script takes about 4 min 59 s (585 words).
- Open without notes. Story first, then the problem from the world down to Albay, then the full title. Greet the panel and give your name only after the title.
- Hold each (pause) for a breath and each (long pause) for three seconds. Never read a slide; point to it and say what it means.
- Slides 1 to 3 (blank, cover, opening prayer) come before the clock. Start timing on slide 4.
- On slide 8, count the three points. On slide 16, present the missing field data as a decision. No apology.
- On slide 23, follow the five beats: the answer, the three points counted again, the next step.
- After "in time," wait three seconds before "Thank you." The PowerPoint speaker notes hold the same script.

Run sheet

Slide	Part	Time	Length
4	Opening: The Problem	0:00 to 0:39	39 s
5	Opening: Albay	0:39 to 1:00	20 s
6	Title and Introduction	1:00 to 1:26	26 s
7	Introduction: Background	1:26 to 1:32	7 s
8	Introduction: The Three Main Points	1:32 to 1:53	20 s
9	RRL/RRS: Related Literature	1:53 to 2:02	9 s
10	RRL/RRS: Related Studies	2:02 to 2:11	9 s
11	Theoretical Framework	2:11 to 2:17	6 s
12	Conceptual Paradigm	2:17 to 2:25	8 s
13	Significance of the Study	2:25 to 2:34	9 s
14	Objectives of the Study / SOP	2:34 to 2:43	9 s
15	Hypotheses	2:43 to 2:54	11 s
16	Scope and Delimitation	2:54 to 3:05	10 s
17	Methodology: Research Design	3:05 to 3:09	4 s
18	Methodology: Population and Sample	3:09 to 3:16	7 s
19	Methodology: Data Gathering Tools	3:16 to 3:21	6 s
20	Methodology: Data Gathering Procedure	3:21 to 3:26	5 s
21	Methodology: Treatment of Data	3:26 to 3:35	9 s
22	Methodology: Ethical Considerations	3:35 to 3:42	7 s
23	Conclusion	3:42 to 4:08	26 s
24	Prototype: Transit Map and Fare	4:08 to 4:16	8 s
25	Prototype: Patient Map and Beds	4:16 to 4:28	11 s
26	Closing and Questions	4:28 to 4:59	31 s

Script

Slide 4 Opening: The Problem

0:00 to 0:39 (39 s)

Walk to the center before speaking. Wait for full silence. Tell the story to the panel, not to the screen.

Two in the morning, somewhere in Albay. A father carries his daughter, who can barely breathe, to the hospital his phone says is nearest. (pause) It has no free bed. (long pause) Worldwide, 4.6 billion people lack essential health services, and nearly four in ten city dwellers lack convenient public transport. In the Philippines, Metro Manila has had open route data since 2013. Albay never has. This August, a Legazpi councilor was still fielding questions on jeepney route structuring.

Slide 5 Opening: Albay

0:39 to 1:00 (20 s)

Read the line on the screen slowly, as if for the first time.

Here, 1.37 million people ride routes no computer can read, at fares no one has mapped, toward hospitals whose free beds no one can see. (pause) A map can show you where to go. Our model shows how to get there.

Slide 6 Title and Introduction

1:00 to 1:26 (26 s)

Slow down for the title. Then greet the panel and introduce yourself.

This is MAP - A: A Landmark-Anchored Timetable-Graph Model of Public Transport: Verified Computation of Accessibility, Its Inequality, and the Fare Matrix from Open Data, with Agent-Reported Hospital Bed Capacity for Ambulance and Patient Routing. (pause) Good morning, honorable panel. I am Dave Justin De Asis, a researcher from Legazpi City Science High School.

Slide 7 Introduction: Background

1:26 to 1:32 (7 s)

Advance, then turn back to the panel. Point to Table 1; do not read it.

Every figure here is public. No one has joined them into one network.

Slide 8 Introduction: The Three Main Points

1:32 to 1:53 (20 s)

Count the three points on your fingers. Give each its own breath.

MAP - A stands on three points. (pause) A transit map that forecasts the time and the fare of every trip. A patient transport map that fits the hospital to the patient. And the bed capacity of every hospital in Albay.

Slide 9 RRL/RRS: Related Literature

1:53 to 2:02 (9 s)

Gesture to the Point column of Table 2.

The literature gives each point its method: exact routing and regulated fares, DOH care levels, and software agents.

Slide 10 RRL/RRS: Related Studies

2:02 to 2:11 (9 s)

Nairobi and Accra mapped their routes, but only after fieldwork, and none tied a trip to a free bed.

Slide 11 Theoretical Framework

2:11 to 2:17 (6 s)

Move your hand down the three rows of Figure 1.

Three gaps, one under each point, each met by a theory.

Slide 12 Conceptual Paradigm

2:17 to 2:25 (8 s)

Sweep a hand across Figure 2, from Input to Output.

Public data go in, proven mathematics does the work, and the three points come out.

Slide 13 Significance of the Study

2:25 to 2:34 (9 s)

Commuters gain the fare, ambulance crews the right hospital, and the health office a bed table in a typhoon.

Slide 14 Objectives of the Study / SOP

2:34 to 2:43 (9 s)

Slow down. This is the sentence the panel will grade.

The objective joins the three points in one model; each of the six questions serves one of them.

Slide 15 Hypotheses

2:43 to 2:54 (11 s)

Lean on the word "before."

Eight hypotheses were fixed before the Albay instance ran. Two are exact by proof; if either fails, the error is in the code.

Slide 16 Scope and Delimitation

2:54 to 3:05 (10 s)

Step toward the panel. Calm and direct, never defensive.

All 18 LGUs. No field data, by design: a model must be built and verified before the field can test it.

Slide 17 Methodology: Research Design

3:05 to 3:09 (4 s)

Formulate, prove, verify, and only then experiment.

Slide 18 Methodology: Population and Sample

3:09 to 3:16 (7 s)

The population is not people. It is every named public landmark in Albay.

Slide 19 Methodology: Data Gathering Tools

3:16 to 3:21 (6 s)

One laptop, no server, and every result checked against independent code.

Slide 20 Methodology: Data Gathering Procedure

3:21 to 3:26 (5 s)

Eight stages, each tagged with the point it serves.

Slide 21 Methodology: Treatment of Data

3:26 to 3:35 (9 s)

One equation per point: the fare of a journey, the fit of a hospital, and its free beds.

Slide 22 Methodology: Ethical Considerations

3:35 to 3:42 (7 s)

No person is studied, and no hospital is contacted until it opts in.

Slide 23 Conclusion

3:42 to 4:08 (26 s)

Five beats. Count One, Two, Three on your fingers, as on slide 8. Pause before the last line.

ANSWER The objective 3:42 to 3:46	Is open data enough? Yes, for all three points.
POINT 1 Transit map 3:46 to 3:50	One: the transit map is exact, 1,018 of 1,018.
POINT 2 Patient map 3:50 to 4:00	Two: the patient search is exact, 5,200 of 5,200. In the test network, five of six starting points had a full nearest hospital.
POINT 3 Hospital beds 4:00 to 4:05	Three: not one of 200 forged bed reports got through.
NEXT STEP Field validation 4:05 to 4:08	(pause) Field validation comes next.

Slide 24 Prototype: Transit Map and Fare

4:08 to 4:16 (8 s)

Gesture from the left phone to the right.

This is the prototype. Ask for a trip, and it returns the time and the fare.

Slide 25 Prototype: Patient Map and Beds

4:16 to 4:28 (11 s)

Hospital mode reads the patient's condition and names the hospital that can take them. The bed dashboard lists every licensed facility in Albay.

Slide 26 Closing and Questions

4:28 to 4:59 (31 s)

Step forward. Slower. Hold eye contact through the last two lines.

Back to that father. (pause) MAP - A will not add one bed to Albay. But once hospitals report, it will tell him which one has a free bed and the right care. (pause) Distance tells you where the hospital is. Access tells you whether it can help in time. (long pause) Thank you. I welcome your questions.